MEDICOVER HOSPITALS
1-95/68 and 1-95/69,Plot no 68 and 69 Sy no 78,Partrika Nagar,Madhapur,
Hyderabad,Telangana, 500081
GST No - 36AAPCS2663H1ZZ

Discharge Summary

Patient Details

Name	MR GOVIND ROY	Age	65Y
Admn. No.	MC35IP26030452	UMR No.	552311000448
Admn. Dt.	10-Mar-2026 14:42	Gender	MALE
Discharge Date & Time	17 Mar 2026 10:33 AM	Mobile No	8919736316
Doctor Name	DR. MEGHANA SUBHASH	Blood Group	
Address	HYD	Ward/Room/ Bed	1ST FLOOR ICCU 2 / 2
Organisation	MEDICOVER STAFF DEPENDENT		

Health Conditions	Code Type	Duration	Remarks

Consultant:
DR. MEGHANA SUBHASH
MD, DM PULMONARY MEDICINE (AMRITA INSTITUTE OF MEDICAL SCIENCES), EDARM (SWITZERLAND)
CONSULTANT INTERVENTIONAL PULMONOLOGIST AND SLEEP MEDICINE SPECIALIST
DEPARTMENT OF PULMONOLOGY
Diagnosis :
ACUTE EXCERBATION OF COPD

TYPE II RESPIRATORY FAILURE

? OBSTRUCTIVE SLEEP APNEA / OBESITY HYPOVENTILATION SYNDROME

SYSTEMIC HYPERTENSION
Chief Complaint :
SHORTHNESS OF BREATH
COUGH
CHEST TIGHTNESS
DROWSINESS
History Of Present Illness :
65 YEAR OLD GENTLEMAN KNOWN HYPERTENSIVE, COPD

PRESENTED WITH COMPLAINTS OF SOB 2-3 SINCE 2 WEEKS AGGRAVATED TO GRADE 4 SINCE 1 DAY ASSOCIATED WITH COUGH WITH EXPECTORATION, CHEST TIGHTNESS, WHEEZING, DROWSINESS . HE WAS INITIALLY EVALUATED AND TREATED AT OUTSIDE CENTRE, REFERRED HERE FOR FURTHER MANAGEMENT.

ON ARRIVAL TO ER
SPO2- 66%@RA

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Admn. Dt. :	10-Mar-2026 14:42	Doctor Name:	DR. MEGHANA SUBHASH

Past History :
SYSTEMIC HYPERTENSION
CHRONIC OBSTRUCTIVE PULMONARY DISEASE

H/O OF HOSPITALIZATION REQUIRING MECHANICAL VENTILATION 1 YEAR AGO
Treatment History :
TAB TELMA AM BD
Allergies :
NO KNOWN DRUG AND FOOD ALLERGIES
Personal History :
SLEEP - ADEQUATE,
APPETITE - DECREASED
BOWEL BLADDER HABITS- REGULAR
DIET- MIXED
Family History :
NOTHING SIGNIFICANT
General Examination :
PATIENT CONSCIOUS , COHERENT , AND OBEYING COMMANDS
NO PALLOR , CYANOSIS , CLUBBING , ICTERUS , LYMPHADENOPATHY
Systemic Examination:
CVS: S1 S2 +
RS: B/L EXPI WHEEZE+
P/A: SOFT, NON-TENDER
CNS: NAD
Course in the Hospital:
A 65 YEARS MALE PRESENTED TO THE HOSPITAL WITH THE ABOVE MENTIONED HISTORY. ALL THE NECESSARY INVESTIGATIONS WERE DONE. ON ARRIVAL TO ER - PATIENT IS TACHYPNEIC , 66% SPO2 ON RA PATIENT SHIFTED TO MICU-1, ABG S/O RESPIRATORY ACIDOSIS , INTIATED NIV , DUE TO NIV FAILURE AND PERSISTANT RESPIRATORY DISTRESS PATIENT WAS INTUBATED ON 11/03/2026. DURING THE STAY IN THE HOSPITAL PATIENT TREATED WITH IV ANTIBIOTICS , NEBULIZED BRONCHODILATORS, ANALGESICS , ANTICOAGULANTS,SYSTEMIC CORTICOSTEROIDS , MUCOLYTICS AGENTS, PPI'S, MECHANICAL VENTILATION AND OTHER SUPPORTIVE MEASURES.2D ECHO S/O NO RWMA OF LV NORMAL LV / RV FUNCTION GRADE I LV DIASTOLIC DYSFUNCTION TRIVIAL MR / NO AR MILD TR / MODERATE PAH NO PE/CLOTS IVC DILATED, USG BEDSIDE ABDOMEN & PELVIS S/O MILD HEPATOMEGALY WITH WAVY CONTOURS. BILATERAL THIN RIM OF PERINEPHRIC FLUID. UMBILICAL HERNIA. BILATERAL MILD PLEURAL EFFUSIONS WITH SUBSEGMENTAL COLLAPSE OF UNDERLYING LUNG PARENCHYMA. AFTER SERIAL ABG MONITORING PATIENT SHOWED CLINICAL IMPROVEMENT AND WAS SUCCESSFULLY EXTUBATED ON 12/03/2026. HE WAS SHIFTED OUT AND CONTINUED WITH SUPPLEMENTAL OXYGEN AND NON INVASIVE VENTILATION. NOW THE PATIENT IS HEMDYNAMICALLY STABLE AND HENCE BEING DISCHARGED WITH FOLLOWING MEDICAL ADVISE, MAINTAINING SPO2 94% WITH 1L/MIN SUPPLEMENTAL OXYGEN AND NOCTURNAL BIPAP.

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Investigation Reports:

ENCLOSED

Preventive Measures:

STRICT USE OF MEDICATIONS ACCORDING TO ADVICED TIMING.
AVOID ANY ACTIVITY THAT INCLUDES EXERTION.
AVOID LIFTING HEAVY WEIGHTS.

Urgent Care Instructions:

IN CASE OF ANY OF THE FOLLOWING :

BREATHLESSNESS
HEMOPTYSIS
WHEEZING
CHEST PAIN
ALTERED LEVEL OF CONSCIOUSNESS
FEVER ABOVE (101F)
NEW ONSET OF PAIN AND WORSENING OF EXISTING PAIN
WORSENING OF ANY OF YOUR SYMPTOMS

ANY OTHER SIGNIFICANT COMPLAINTS
HOSPITAL CALL CENTER: 040-68334455
PULMONOLOGY EMERGENCY - 9063042906

PATIENT AND ATTENDERS WERE EXPLAINED ABOUT DISCHARGE ADVICE AND URGENT CARE INSTRUCTIONS IN THE LANGUAGE UNDERSTOOD BY THEM

Follow Up Instructions:

REVIEW AFTER 1 WEEK IN PULMONOLOGY OPD(WITH DR. MEGHANA SUBHASH)

Transfer/Shiftout/Discharge Details:

BEING DISHCHARGED IN HEMODYNAMICALLY STABLE CONDITION

Advised:

BIPAP WITH IPAP:EPAP OF 14:6 DAYTIME INTERMITTENT, NIGHT CONTINUOUS FOR 6-8 HOURS/NIGHT
INHALATION OXYGEN AT 1-2L/MIN VIA NASAL PRONGS FOR 16-18 HOURS/DAY ATLEAST

Medications Advised :

Medicine Name	Dose/Volume/Tab	When	Route	Frequency	Duration	Instruction
DOXY 100MGTABLET (GN:)		After Food		1-0-1	5/Days	
ELIQUIS 5MG TABLET (GN:)		After Food	ORAL	1-0-1	15/Days	
MONTEK FX TABLET (GN:)		After Food	ORAL	0-0-1	10/Days	
TELMA AM 40MG TABLET (GN:)		After Food		1-0-0	15/Days	

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SOMPRAZ 40 TABLET (GN:)		Before Food	ORAL	1-0-0	10/Days	
SYP DUPHALAC (GN:)	30 ML	After Food	ORAL	SOS	5/Days	
TAB PULMOCLEAR (GN:)	100/600 MG	After Food	ORAL	1-0-1	10/Days	
NEB FORACORT (GN:)	0.5 MG		INHALATIONAL	1-0-1	0/Till Further Advice	
NEBSMART G (GN:)			INHALATIONAL	1-0-1	0/Till Further Advice	

Dietary Advised:

AS PER DIETICIAN ADVICE

HOME HEALTH CARE:

PLEASE CALL 040-68334455

For THE FOLLOWING HOMECARE SERVICES

1. Doctor Consultation
2. Nursing
3. Sample Collection
4. Physiotherapy
5. Basic Medical Procedures

DR. MEGHANA SUBHASH||Consultant Interventional
Pulmonologist And Sleep Medicine Specialist